

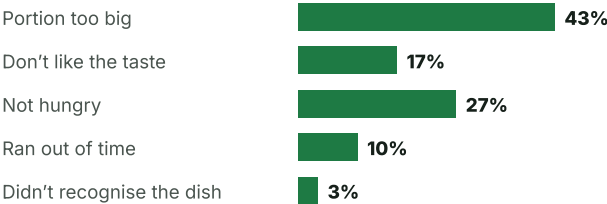
PlateLoop survey: key findings

Students, canteen and kitchen staff, hospital staff and office workers.

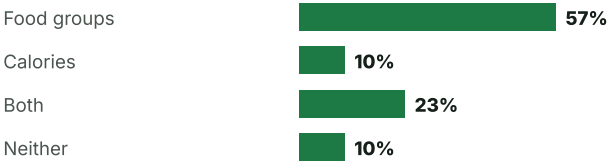
Demo data. These results come from synthetic responses (students 30, kitchen 8, hospital 6, office 6) generated to demonstrate the analysis. They are not real survey results and should not be quoted as evidence; with groups this small, each response moves a percentage by several points. The questionnaires and analysis workbook are ready for real responses.



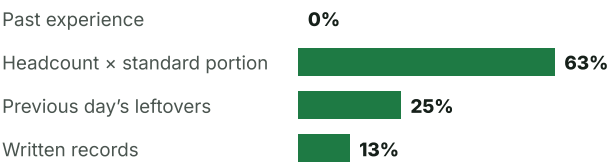
Students: main reason for leaving food



Students: what they would rather see



Kitchen staff: how quantities are decided



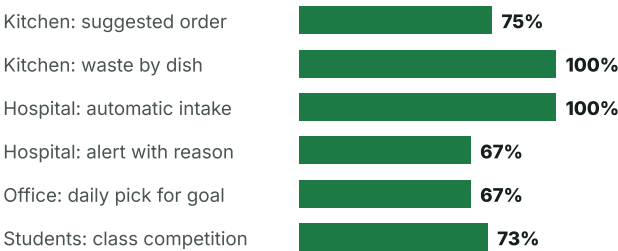
Hospital staff: when a patient who stops eating is noticed



Office workers: share of meal data with employer



Share who agree (4–5) by statement



Figures computed from the demo responses in plateloop-survey-analysis-DEMO.xlsx.

Findings and what they mean for PlateLoop

- The most common reason for leaving food is 'Portion too big' (43%); 47% would take a smaller portion if they could ask easily.
→ The before scan and right-portion rewards in Loopi: a small plate finished counts fully.
- When seeing what they ate, 57% would rather see food groups and 10% calories.
→ Loopi shows food groups by default; calories stay optional.

-
- 3** Interest in scanning trays to feed a virtual pet averages 4.0 out of 5; 67% agree.
→ The Loopi pet as the reason students scan.
-
- 4** 43% of students are comfortable with face sign-in.
→ Keep the class-and-number alternative; face sign-in stays optional.
-
- 5** 63% of kitchen staff decide quantities by experience or headcount; 13% know accurately which dishes come back.
→ Kitchen dashboard: waste by dish and a forecast from real eating data.
-
- 6** Kitchen staff estimate 22% of cooked food is thrown away and spend 32 minutes a day planning quantities.
→ A suggested order for tomorrow saves planning time and cuts over-preparation.
-
- 7** A suggested order is rated useful by 75% and waste by dish by 100%; 50% worry about extra workload.
→ Measurement is automatic: no forms or manual entry for kitchen staff.
-
- 8** 17% of hospital staff trust their intake records; documenting intake takes 31 minutes a shift.
→ Loopi Care records every tray automatically.
-
- 9** Staff say a patient who stops eating is noticed the same day in 33% of cases.
→ Nurse-station alerts after poor meals, the same day.
-
- 10** Automatic intake records are rated useful by 100%; 50% say patients would benefit from choosing tomorrow's meals.
→ Loopi Care: per-tray intake and patient meal choice.
-
- 11** 33% of office workers know the nutrients in their lunch; 50% feel an afternoon energy dip often or daily.
→ Loopi Work: nutrients per lunch and a 3 pm energy check-in.
-
- 12** 33% would pre-order lunch to skip a typical 10-minute queue.
→ Pre-ordering in Loopi Work; kitchens cook to orders.
-
- 13** 17% would share meal data with their employer; 50% would share anonymous totals only.
→ Private by default: employers see team totals, never names.